

Standard Operating Procedure

Safe Prescribing — Allergy Check, Completeness and Decision Support

«Hospital Name» — Supports MOM.4.a, MOM.4.b (CORE), MOM.4.c (Stop-Work), MOM.4.d

SOP No.	[Hospital to define]	Version	1.0
Effective Date		Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» prescribes rationally and safely — checking allergy history before every prescription, writing every prescription with all required elements, and using decision-support tools where available.

2. Scope

Applies to every outpatient and inpatient prescription written at «Hospital Name».

3. Responsibility

Prescribing Clinician: checks allergy/ADR history, writes a complete prescription, and follows the Code of Medical Ethics-2002.

Pharmacist: holds non-compliant prescriptions and confirms decision-support alerts are actioned.

4. Procedure

4.1 Allergy and adverse-reaction check

1. Ascertain drug allergies and previous adverse drug reactions before every prescription — this step comes first, before the medication is decided.
2. Document the allergy/ADR check in the patient record, whether the finding is positive or negative.

4.2 Writing the prescription

1. Include all seven minimum elements on every prescription: patient name, age/weight (paediatric), date, medication name, dose, route, frequency, and duration, plus the prescriber's name and signature.
2. Write in capital letters where handwritten, and use only approved, non-error-prone abbreviations.

Common error-prone abbreviations to avoid include "U" for units (misread as a zero, causing a 10-fold overdose), "QD"/"QOD" (mistaken for each other), a trailing zero as in "1.0 mg" (misread as 10 mg), and "MgSO4"/"MSO4" (confused with each other) — see the ISMP List of Error-Prone Abbreviations, Symbols, and Dose Designations (ismp.org).

3. Where a prescription is missing a required element or uses a non-compliant abbreviation, the pharmacist holds it and contacts the prescriber for correction before dispensing.

4.3 Decision support

1. Use the available clinician decision-support mechanism — electronic or physical — to check drug interactions, food-drug interactions, therapeutic duplication, and dose adjustments.

2. Document the alert and the action taken whenever a decision-support alert fires.

5. Stop-Work Authority

Do not prescribe without first checking drug allergy and adverse drug reaction history — for every patient, every time.

The pharmacist holds any prescription with a missing required element or a non-approved abbreviation until it is corrected — dispensing does not proceed on a held prescription.

Raising a missed allergy check or an incomplete prescription is never a disciplinary matter, regardless of the prescriber's seniority.

6. Records

Supported by Records 1 through 5 of the MOM.4 Required Records pack.

7. Related Documents

- Policy on Safe and Rational Prescription of Medications (HCO/MOM/POL/04) — MOM.4 policy
- SOP — Verbal Order Process (MOM.4.f)
- SOP — Medication Reconciliation (MOM.4.e)

8. Review

Reviewed annually, and immediately after any allergy-related adverse event.

9. References

- National Medical Commission (adopting the Indian Medical Council). (2002). Code of Medical Ethics — Professional Conduct, Etiquette and Ethics Regulations. <https://www.nmc.org.in/wp-content/uploads/2017/10/Ethics-Regulations-2002.pdf>